

**Assessing the Impact of
Canada's National Housing Strategy**
*An Equity-Based Data Stewardship Framework for New
Housing Supply Initiatives*

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Introduction

Launched in 2017 and billed as “the largest and most ambitious federal housing program in Canadian history” (Government of Canada 2017), the National Housing Strategy (NHS) is the federal government’s ten-year, \$80+ billion plan¹ to ensure all Canadians have access to housing that meets their needs and that they can afford. The strategy is composed of an array of policies, programs, and commitments—set against headline goals of a 50% reduction in chronic homelessness² and the removal of 530,000 households from housing need—that target various issues (e.g., homelessness, accessibility, etc.) and points along the housing continuum (e.g., transitional housing, affordable rental housing, home ownership, etc.).

Among the programs that compose the NHS and forming a cornerstone of the strategy, are a number of unilateral federal initiatives, the purpose of which is to expand the affordable housing stock. They include the National Housing Co-Investment Fund (NHCF), the Rental Construction Financing Initiative (RCFI), the Affordable Housing Innovation Fund (AHIF), the Federal Lands Initiative (FLI) and, more recently, the Rapid Housing Initiative (RHI). Taken together, these new supply initiatives have received over half of the total investment committed as part of the NHS, marking what is framed in the strategy as a re-engagement by the federal government in the provision of affordable housing after over four decades of retreat.

The NHS also signifies a normative shift in federal housing policy, given its grounding in a human rights-based approach. Passed in 2019 in fulfilment of a core NHS commitment to base the strategy and commitments in legislation, the National Housing Strategy Act (S.C. 2019, c. 29, s. 313) establishes requirements regarding the use of federal policy to further the progressive realization of the right to adequate housing in Canada. One way the NHS seeks to operationalize this responsibility, in line with international human rights law, is by committing to prioritizing the needs of 11 vulnerable groups who are more likely to experience housing need than others. The NHS was also developed using a Gender-Based Analysis Plus (GBA+) lens and, as indicated in the strategy, this framework will be used to shape monitoring and evaluation efforts as well.

Six years into implementation, however, both the current impact and the future potential of the NHS remain open questions. Housing experts have spilled much ink to outline the shortcomings and limited effectiveness of the NHS to date, not only in terms of expanding affordable housing supply, but also in meeting the needs of vulnerable groups. Analyzing the NHS from a rights-based and GBA+ perspective, Schwan and Ali (2021) find that the affordability criteria for both the NHCF and the RCIF are not set at a depth that will ensure that new supply can meet the needs of deeply marginalized women and gender diverse people, and that program application requirements impose considerable barriers for grassroots and non-profit housing providers with knowledge of the unique needs and experiences of marginalized groups, undermining their ability to benefit from—and thus serve vulnerable groups through—NHS funding. More broadly, Lee (2022) suggests that though the NHS has delivered support for existing social housing and has provided a necessary investment in initiatives targeting homelessness, the strategy has not delivered on its promise to expand affordable supply, and has failed to challenge—and has indeed entrenched through the provision of low-cost loans to private developers—Canada’s overreliance on market housing.

¹ Initially, the plan set out a \$40 billion budget; however, in subsequent years, the federal government has purportedly doubled this investment through subsequent budgets, with the total now sitting at over \$82 billion (Office of the Parliamentary Budget Officer 2023).

² As noted by Pomeroy (2021b) the Government of Canada has since revised its commitment in this area, and is now aiming for a 100% reduction (i.e., the eradication of chronic homelessness).

Some observers, such as the National Right to Housing Network (NRHN), also question the rigour of evaluation and reporting efforts with regard to the NHS, noting that the focus of such efforts has been on general outcomes—such as number of units created or families housed—with little analysis of the contribution of NHS investments to achieving human rights goals (Biss and Raza 2021, 44). Perhaps the largest barrier to more granular analysis, evaluation, and reporting in accordance with human rights obligations is a paucity of data. At present, there exists no systematic process for gathering basic information about the impact of investments, such as details of who is being housed as a result of NHS programs, nor do data and metrics exist that would enable equity-driven assessments of progress towards progressive realization—that is, data on the groups whom new supply initiatives are, and are not, benefitting, with what impact, and whether beneficiaries belong to groups in the greatest need. When considering the linchpin objective of expanding affordable housing for vulnerable groups, the Auditor General of Canada has found that the Canada Mortgage and Housing Corporation (CMHC) cannot verify which vulnerable groups are benefitting from NHS initiatives (Office of the Auditor General of Canada 2022). Maintaining the status quo will undermine the success and promise of the NHS—if no initiative is taken to close the gaps, Canada will be unable to uphold its commitments under the NHSA.

Commissioned by the Canadian Housing Evidence Collaborative (CHEC) as part of the Affordable Housing Supply Monitor Initiative, this report sets out an equity-based data stewardship framework for affordable housing supply in Canada. The report is structured around two objectives. The first is to identify and delineate an approach, grounded in best practices in equity and intersectionality, that will enable partners to both collect data on new housing supply created through NHS investments, as well as begin to use this information to evaluate the extent to which the NHS is achieving its rights- and equity-based goals. The second objective is to offer a set of practical insights that can support movement towards more equitable stewardship of housing data and information in Canada—particularly that which concerns vulnerable groups—including through the empowerment of communities to play a central role in tracking and evaluating how the right to housing is being upheld over time. Overall, this work contributes a piece of the necessary foundation for the federal government to be able to report on, and for researchers and communities to be able to hold it to account for, obligations related to the progressive realization of the right to housing in Canada.

The report is structured in four parts. Section One provides background information on the right to housing, gives an overview the NHS approach and the new housing supply initiatives advanced through the strategy, and summarizes current data gaps. Focusing in on the 11 vulnerable groups identified in the NHS, Section Two uses the expansive definition of adequate housing, as established in international human rights law, to conceptualize the multi-level factors that influence access to housing for diverse people in Canada, in intersectional perspective. Section Three presents a synthesis of the current knowledge base on equitable data stewardship, and highlights guiding concepts and wise practices. Drawing on the conceptual work and synthesis in Sections Two and Three, Section Four sets out and explains an Equity-Based Data Stewardship Framework, spanning elements such as foundational principles, institutional and societal context, community engagement, question development, collection approach, and plans for data analysis, ownership, and storage. The Framework also offers a number of practice-based recommendations.

1. Background on the NHS: Rights, Rental Supply, and Progressive Realization

The development of an equity-based data stewardship framework for NHS affordable supply initiatives rests on first understanding the context within which the NHS was created, the commitments outlined in the strategy in terms of the right to adequate housing, and the various unilateral federal initiatives focused on new construction which form a key part of Canada’s approach to progressive realization. This section

outlines these elements, connecting the right to adequate housing and progressive realization obligations to the need for data collection.

1.1. The Right to Adequate Housing

Among the most notable aspects of the NHS is its grounding in the language and principles of international human rights law. Specifically, the National Housing Strategy Act—which was adopted in 2019, two years after the launch of the NHS—reinforces the rights-based approach woven throughout the strategy, in partial fulfilment of Canada’s obligations under human rights treaties. Internationally, the right to adequate housing has long been recognized as one aspect of the universal right to an adequate standard of living, including in article 11 of the 1966 International Covenant on Economic, Social and Cultural Rights (ICESCR). As clarified in the general comments of the United Nations (UN) Committee on Economic, Social and Cultural Rights (CESCR), the right to housing should be interpreted not merely as having access shelter, but instead as “the right to live somewhere in security, peace and dignity” (1991). Specifically, the UN CESCR, in its General comment No. 4, indicates that the concept of adequacy takes on particular significance in the context of housing rights, where adequate housing reflects several aspects, which apply across various economic, cultural, climatic, ecological, and other contexts (UN CESCR 1991). That is, adequate housing is that which meets basic conditions spanning legal security of tenure; availability of services, materials, facilities, and infrastructure; affordability; habitability; accessibility; location; and cultural adequacy.³ Table 1 summarizes each dimension of adequacy, as outlined by the UN CESCR.

Table 1: Dimensions of Housing Adequacy in International Human Rights Law

Dimension	Explanation
Legal security of tenure	All persons should possess a degree of security of tenure which guarantees legal protection against forced eviction, harassment, and other threats.
Availability of services, materials, facilities, and infrastructure	Must contain facilities essential for health, security, comfort and nutrition. All persons should have sustainable access to natural and common resources, safe drinking water, energy for cooking, heating and lighting, sanitation and washing facilities, means of food storage, refuse disposal, site drainage, emergency services.
Affordability	Personal or household financial costs associated with housing should be at such a level that attainment and satisfaction of other basic needs are not threatened.
Habitability	Housing must provide the inhabitants with adequate space and protect them from cold, damp, heat, rain, wind or other threats to health, structural hazards, and disease vectors. Physical safety of occupants must be guaranteed as well.
Accessibility	Housing must be accessible to those entitled to it, and disadvantaged groups must be accorded full and sustainable access to adequate housing resources. Such groups as the elderly, children, the physically disabled, the terminally ill, HIV-positive individuals, persons with persistent medical problems, the mentally ill, victims of natural disasters, people living in disaster-prone areas and other groups should be ensured some degree of priority consideration in the housing sphere.
Location	Adequate housing must be in a location which allows access to employment options, health-care services, schools, childcare centres and other social facilities.
Cultural adequacy	The way housing is constructed, the building materials used and the policies supporting these must appropriately enable the expression of cultural identity and diversity of housing. Activities geared towards modernization in the housing sphere should ensure that cultural dimensions of housing are not sacrificed, and that, inter alia, modern technological facilities, as appropriate are also ensured.

³ This broad interpretation of adequacy also signals that realizing and upholding the right to adequate housing will require a mix of legal, policy, and programmatic instruments beyond investments in new supply, including legal protections, tax-based measures, and interventions to address financial systems and actors (Biss et al. 2022, 4).

The concept of progressive realization underpins the responsibilities of states in relation to economic, social, and cultural rights, and as a result plays an important role in how housing rights are realized in a given jurisdiction. Set out in article 2(1) of the ICESCR, the progressive realization obligation requires that states “take steps, individually and through international assistance and co-operation, especially economic and technical, to the maximum of its available resources, with a view to achieving progressively the full realization of the rights recognized [...] by all appropriate means, including particularly the adoption of legislative measures.”⁴ This both reflects an acknowledgement that not all aspects of the right to adequate housing can or may be realized immediately, given resource constraints and the time required to extend the right to everyone, as well as maintains state accountability for demonstrating that “in aggregate, the measures being taken are sufficient to realize the right for every individual in the shortest possible time in accordance with the maximum of available resources” (UN CESCR 1991).⁵ In periodic reviews conducted by the CESCR, the standard of “maximum available resources” has been assessed through a consideration of budgetary allocations within the context of state goals and targets; the expert opinion of UN agencies, organizations, advocacy and civil society groups, and affected communities and rights holders; and inter-country comparisons (Biss et al. 2022, 56). Progressive realization also involves particular obligations in relation to vulnerable and marginalized groups, requiring that “state parties [...] give due priority to those social groups living in unfavourable conditions by giving them particular consideration” (UN CESCR 1991). This means implementing positive measures that offer preferential treatment, including in the form of targeted measures, to vulnerable and disadvantaged groups, whose protection must be ensured even in the case of resource constraints (Chenwi 2010).

1.2. The National Housing Strategy Act and the NHS Approach

Though Canada ratified the ICESCR in 1976, the federal government had made no statutory commitment to upholding the right to housing prior to the passage of the National Housing Strategy Act in 2019. Overall, the NHSA acknowledges the right to adequate housing as affirmed in international law; outlines the responsibility of the Canadian government to pursue the progressive realization of this right through federal housing policy; and further commits the government to the development and maintenance of a national housing strategy that reflects four human rights-based principles. These principles are to: (1) set out a long-term vision for housing in Canada that recognizes the importance of housing in achieving social, economic, health, and environmental goals; (2) establish national goals relating to housing and homelessness and identify related priorities, initiatives, timelines, and desired outcomes; (3) focus on improving housing outcomes for persons in greatest need; and (4) provide for participatory processes to ensure the ongoing inclusion and engagement of civil society, stakeholders, vulnerable groups, and persons with lived experience of housing need, as well as those with lived experience of homelessness.

To this end, the NHS sets a number of national housing targets, including cutting chronic homelessness by 50%, removing 530,000 families from housing need, renovating and modernizing 300,000 homes, and building 160,000 new homes. Further, to fulfil the principle of improving outcomes for persons in greatest need, the NHS identifies “housing for those in greatest need” as one of six priority areas for action, and notes 11 subgroups of vulnerable people in Canada whose needs will be addressed through the strategy (Canada Mortgage and Housing Corporation 2023). They include women and children fleeing domestic violence; seniors; young adults; Indigenous peoples; people with disabilities; people dealing with mental health and addiction issues; veterans; the 2SLGBTQIA+ community; racialized groups; recent

4 International Covenant on Economic, Social and Cultural Rights, *adopted* 16 Dec. 1966, G.A. Res. 2200 (XXI), U.N. GAOR, 21st Sess., U.N. Doc. A/6316 (1966), 993 U.N.T.S.3 (*entered into force* 3 Jan. 1976).

⁵ Importantly, progressive realization does not apply to some components of the right to adequate housing, such as non-discrimination (Office of the United Nations High Commissioner for Human Rights 2014, 30), as well obligations to provide legal security of tenure (including in legally protecting against forced evictions), and to monitor the progressive realization of the right to housing in the state concerned (Chenwi 2010, 31).

immigrants, especially refugees; and people experiencing homelessness. The NHSA also establishes a National Housing Council to oversee implementation, and a Federal Housing Advocate, whose mandate involves monitoring NHS progress and assessing its impact on vulnerable groups.

1.3. NHS Supply Initiatives: New Rental Construction

Fundamental to the progressive realization of the right to housing in Canada is the creation of new housing supply. On this priority, the NHS reverses what has been characterized as “long trend of federal devolution and disentanglement” (Pomeroy 2021a, 10) on the housing file through an explicit re-engagement of the federal government and CMHC in the design and delivery of programs to support the construction of affordable housing. Accounting for over half of all funding allocated through the strategy, the linchpin programming advanced under the NHS consists of a suite of unilateral federal initiatives administered by CMHC, with the goal of creating 100,000 new housing units.⁶ Outlined below, these programs include the National Housing Co-Investment Fund (NHCF), the Rental Construction Financing Initiative (RCFI), the Affordable Rental Housing Innovation Fund (ARHIF), the Federal Lands Initiative (FLI), and the Rapid Housing Initiative (RHI).

National Housing Co-Investment Fund

Originally launched in 2018 as the largest budget item of all NHS initiatives at \$13.17 billion,⁷ the NHCF consists of low-interest and forgivable loans, as well as financial contributions, to support the renovation, repair, and construction of new affordable and community housing, with targets for both repaired units (240,000 units) and new construction (60,000 units). Within these streams, the NHCF also establishes targets for unit type and population group, including shelter beds for survivors of gender-based violence, units for seniors, and units for people with developmental disabilities (Canada Mortgage and Housing Corporation 2021b). Eligible projects include community and affordable housing; urban indigenous community housing; mixed-use market/affordable rentals; new construction and/or conversion from a non-residential use to affordable multi-residential; shelters; and transitional and supportive housing. The Fund has two streams: funding for new construction and renovation funding.

The New Construction stream is focused on supporting the development of mixed-income, mixed-tenure, and mixed use (including affordable) housing that meets further criteria related to accessibility, energy efficiency, and social inclusion. To qualify for funding, the applicant—which can be a municipality, province or territory, Indigenous government or organization, entity from the community housing sector, or a private actor, such as a developer—must receive support from another level of government. Applications are prioritized for funding based on achievement of NHS outcomes; provincial and territorial input; fulfilment of criteria for affordability, energy efficiency, and accessibility; proximity to transit, amenities, and community supports; collaboration and partnerships; social inclusion; and support for federal priority groups. To meet affordability criteria, rents for at least 30% of the units in the building must be offered at less than 80% of Median Market Rent (MMR) for the area, and this level must be maintained for at least 20 years. Accessibility is achieved if 20% of the units within the project meet or exceed accessibility standards, and if common areas are barrier-free or apply full universal design. Projects must also demonstrate that they will achieve energy consumption reductions relative to standards set out in the National Energy Code for Buildings.

⁶ A parallel focus within the NHS has been on programs delivered by provinces and territories under bi-lateral agreements with the federal government, which require cost-matching funding. These programs include the Canada Community Housing Initiative, the Provincial-Territorial Priorities Fund, and the Canada Housing Benefit. While important, these programs are not discussed in this paper, as the focus of the analysis is on the impacts of unilateral federal initiatives focused on new construction, and the data and information required to conduct such assessments.

⁷ As noted by Pomeroy (2021b), the NHS originally announced the NHCF at \$15.9 billion; however, this included the Rental Construction Financing Initiative, which has since been set on its own and expanded.

As of December 2022, CMHC had committed support to create 28,985 of the 60,000 target units under the new construction stream, 19,271 of which are affordable; 1,123 of which are shelter spaces or transitional units for survivors of gender-based violence; 5,925 of which are for seniors; and 845 of which are for people with developmental disabilities (Canada Mortgage and Housing Corporation 2022). Issues and concerns raised in relation to the NHCF to date, as summarized in Pomeroy (2021b), include a greater emphasis on and weighting of ancillary outcomes, at the expense of affordability; unintended overlap with and dependence on provincial and territorial contributions; skewed allocation of funds; an onerous application and approval process, which may ultimately deter applicants, particularly non-profits; duplication of efforts under other social housing retrofit streams; and omission of support for acquisition.

Rental Construction Financing Initiative

The RCFI is effectively a rental construction stimulus program, through which the CMHC provides loans, worth up to 100% of cost, to private developers for the construction of purpose-built market rentals (and in some cases mixed-use developments), with an overall goal of creating 71,000 new rental units. Introduced in 2016 in advance of the NHS, the RCFI has since received a significant infusion of funds and now constitutes a “centrepiece” of the strategy (Pomeroy 2021c, 1), representing a \$25.75 billion investment and accounting for 85% of new funding under the expanded NHS (Beer et al. 2022, 13). RCFI loans are low in cost and offer recipients favourable terms, including a 10-year term and fixed interest rate. Eligible borrowers include municipalities, non-profits, and for-profit developers. Though the focus of the RCFI is not on creating affordable housing per se, the projects financed through RCFI funding are expected to offer some units with affordable rents. Specifically, the project must fulfil affordability criteria, whereby the total residential rental income of the project must be at least 10% below gross achievable income, and a minimum of 20% of the units must be affordable, with rents at or below 30% of median household income reported by census families in the subject market.⁸ Projects must also meet minimum energy efficiency, emissions, and accessibility standards. Prioritization scoring spans financial viability, affordability, energy efficiency, and accessibility metrics, as well as fostered collaboration and transit oriented eligibility.

As of December 2022, CMHC had committed \$13.93 billion in loans towards the construction of 39,682 units through the RCFI, 27,750 of which are expected to be below market rent (Canada Mortgage and Housing Corporation 2022). It is important to note, however, that the definition of affordability applied to the RCFI program is both inconsistent with those used elsewhere throughout the NHS (in the NHCF, for example), as well as poorly constructed as a measure of affordability. When considering the RCFI’s affordability standard, both the reliance on median income—which is heavily weighted by owners, whose earnings are higher than those of renters, and further inflated given exclusion of households not in census families—as well as the uniformity of the standard across unit types, undermine its usefulness as a metric (Beer et al. 2022). As Pomeroy (2021b, 17) calculates, using 30% of median household income results in rent levels that, in all but two cities in Canada, are well above 150% of average market rents.

Affordable Housing Innovation Fund

Much smaller than the NHCF and RCFI, the AHIF is a \$759.1 million program through which CMHC extends financial support to projects that represent innovative approaches to delivering affordable housing, considering three types of innovation: transformational, breakthrough, and incremental. For example, funding can be attained to develop and test innovations that incorporate resource and operating

⁸ Research conducted for the National Housing Council (see Beer et al. 2022, 26) confirms that CMHC determines affordability relative to median income of families, not households. This means that only census families—that is, households led by married or common-law couples, as well as lone-parent households—are considered when calculating median income. The exclusion of people who live with roommates and those who live alone has the effect of increasing median income, thereby weakening the standard of affordability.

efficiencies, to leverage new partnerships, or to encourage participation of diverse stakeholders. The Phase 1 target for the AHIF was 4,000 affordable units. In Phase 2, the goal increased to 10,800 units. As of December 2022, CMHC had committed \$208.30 million to support the creation of 19,100 units through the AHIF, of which 16,572 will be affordable (Canada Mortgage and Housing Corporation 2022).

Federal Lands Initiative

The Federal Lands Initiative represents a total investment of \$200 million and intended to support the transfer of surplus federal lands and buildings to be developed or renovated for use as affordable housing, with a target of creating 4,000 housing units. Eligible properties for acquisition through the fund are identified by federal government departments and Crown corporations (e.g., CMHC, Public Services and Procurement Canada, Employment and Social Development Canada, and Canada Lands Company) and then transferred to eligible proponents at a discount or no cost.

As of December 2022, CMHC had committed \$117.81 million through the FLI to support the creation of 3,916 units, of which 1,684 will be affordable (Canada Mortgage and Housing Corporation 2022). Each project must commit to offering 30% of units at less than 80% of MMR for a minimum of 25 years, as well as 20% of units that meet accessibility standards, with all common areas being barrier free. A 2021 evaluation of the program indicated that, of projects approved, 11% of units were intended to be dedicated, and 12% were intended to be committed, to a specific vulnerable priority group (Canada Mortgage and Housing Corporation 2021a).

Rapid Housing Initiative

Though not initially a part of the NHS, the Rapid Housing Initiative was launched during the COVID-19 pandemic in response to immediate need for expanded affordable supply, and now forms a key component of the new construction initiatives under the NHS. A total of \$2.5 billion was committed through Rounds 1 and 2 of the RHI, and the program has been extended for another two years through a third round of funding, worth \$1.5 billion. Of the unilateral funding initiatives, the RHI is the most explicitly targeted at supporting the housing needs of vulnerable groups. Composed of a series of funding rounds, the RHI aims to create new permanent affordable housing units through capital contributions to applicants who commit to the rapid construction, conversion, or rehabilitation of existing buildings to affordable housing to serve NHS priority populations in deep housing need. Eligible applicants include provinces, territories, municipalities, Indigenous governing bodies, and non-profits (through the project stream) and pre-determined municipalities (in the cities stream). All projects funded under the RHI must create housing for NHS priority groups in severe housing need and all units must meet affordability criteria (i.e., rents less than 30% of income) for a minimum of 20 years. Further, projects are prioritized for funding based on how quickly they can be completed and reach occupancy thresholds: eligibility is based on projects being completed within 12 months.

The first round of the RHI provided \$1 billion of funding to projects to support those in severe housing need, specifically people at risk of or experiencing homelessness and people living in temporary shelters because of the COVID-19 pandemic, while the second round delivered an additional \$1.5 billion to address urgent housing needs of vulnerable Canadians. As of December 2022, Rounds 1 and 2 of the Rapid Housing Initiative had supported the development of 10,249 new deeply affordable housing units, 4,246 of which are for Indigenous peoples, 2,522 of which are for people experiencing or at risk of homelessness; and 1,301 of which are for seniors (Canada Mortgage and Housing Corporation 2022).

Summary

Overall, the unilateral federal supply programs highlighted above target the construction of 161,800 new rental units, a considerable proportion of which will be considered affordable by some metric, and some of which will be specifically designed with target populations, including NHS priority groups, in mind.

Table 2 summarizes these initiatives based on level of investment, target construction of new units, committed new units, target groups, and affordability metric employed. Given the range of eligible applicants, project types, and target groups across these programs, monitoring and evaluation efforts—including data and information collection and stewardship—will require a multi-pronged strategy that involves diverse groups, including various levels of government (and agencies), non-profit and community housing groups, and Indigenous organizations, and which accounts for a range of barriers, complexities, and aspects of housing adequacy.

Table 2: Unilateral Federal Investments in New Rental Construction through the NHS

	Investment	Target (New Units)	Committed (New Units)	Target Groups	Affordability Metric
NHCF	\$13.17B *for both streams	60,000	28,985 (19,271 affordable)	Projects targeting NHS priority groups given first consideration	At least 30% of project units must be $\leq$ 80% of MMR for area for 20 years
RCFI	\$25.75B	71,000	39,682 (27,750 “below market rent”)	Not specified	Res. rental income must be 90% or less of gross achievable; min. 20% units must be affordable (rents $\leq$ 30% of median family income in CMA)
AHIF	\$759.1M (for all projects in phases 1 and 2)	14,800	19,100 (16,572 affordable) *phase 1	Not specified	Defined based on criteria of municipality in which project is located (or province or CMHC). Must be affordable for min 10 years.
FLI	\$200M	4,000	3,916 (1,684 affordable)	Not specified	At least 30% of units must be $<$ 80% of MMR, for min. 25 years
RHI	\$2.5B (R1,2) \$1.5B (R3)	7,500 4,500	10,249 (all affordable)	Targeted to people in NHS priority groups + in severe housing need; 25% of round 3 funds for women-focused projects	100% of units must meet affordability criteria for 20 years (i.e., must pay $<$ 30% of before-tax income or shelter component of IA for housing costs)

1.4. Progressive Realization and Data Needs

As affirmed in guidance issued by the UN Special Rapporteur on the Right to Adequate Housing, the institution of independent monitoring and accountability mechanisms is fundamental to states upholding their obligation to progressively realize the right to adequate housing. Specifically, with regard to housing strategies implemented in support of progressive realization, “states should provide for independent monitoring of progress in meeting goals and timelines, establish procedures through which affected communities can identify systemic issues affecting the realization of the right to housing, and ensure effective responses” (UN Human Rights Council 2020). More broadly, and as noted in Guideline 15 – Ensure effective monitoring and accountability mechanisms:

Monitoring progress on the implementation of the right to housing should be focused on assessing compliance with the obligation to progressively realize the right. It

should include the collection of qualitative and quantitative data related to dignity and the experience of rights holders with respect to all aspects of the right to housing, including security of tenure, availability of services, affordability, habitability, accessibility, location, cultural adequacy, homelessness and evictions. Statistical and qualitative information with appropriate safeguards should also be collected on the housing circumstances of groups facing systemic discrimination, on structural barriers to housing and on the outcomes of measures taken to address such barriers. Major trends and emerging challenges should also be identified (Ibid).

The federal government is now over half way through the ten-year NHS implementation period, and as a result, there is a growing need for a robust data collection and reporting strategy to support the monitoring of progress on the implementation of the right to housing. This is of particular importance in terms of upholding obligations to vulnerable groups. How has Canada fared to date in collecting data and information regarding the impacts of its programs and investments, including for priority groups? What have reporting efforts looked like? What data, information, and reporting gaps remain?

NHS New Affordable Rental Supply Initiatives

Experts have been critical of data collection and outcome reporting on NHS programs and investments to date. For example, Pomeroy (2021b) argues that, despite the publication of a number of progress reports, information provided by CMHC has been neither clear nor transparent, and lacks detailed information, often reflecting only broad counts of the units created, without mention of affordability standards, how many units meet program criteria, or who ultimately benefits from investments. Further, the National Housing Rights Network has noted that NHS programs are not accompanied by monitoring strategies that collect and employ disaggregated data to measure outcomes and impacts for priority populations, undermining Canada's ability to track progress towards human rights obligations; the associated reliance on population-level data prevents analyses which can isolate the particular impacts of NHS programs from those attributable to other dynamics, trends, and programs (Biss and Raza 2021, 79). Indigenous organizations in Canada, including the Assembly of First Nations, Inuit Tapiriit Kanatami, the Métis National Council, Native Women's Association of Canada, Congress of Aboriginal Peoples, the National Association of Friendship Centres and the Indigenous Caucus of the Canadian Housing and Renewal Association, have also expressed the need to fill current data gaps while also ensuring data sovereignty (Office of the Federal Housing Advocate 2022, 18). With particular reference to new rental supply and construction programs, it does not appear as though CMHC is implementing monitoring and information collection strategies necessary to assess who is behind the doors of new units—let alone to understand the resultant housing experiences of NHS priority groups.

The above criticisms were recently confirmed by the Auditor General, whose 2022 audit found that CMHC did not know who was benefiting from NHS initiatives or if target groups were actually occupying housing supported by its initiatives (Office of the Auditor General of Canada 2022). In their report of findings, the AG further confirmed that CMHC was not collecting the disaggregated demographic data required to measure the impact of strategy initiatives on specific groups within vulnerable populations, nor was the corporation measuring outcomes for vulnerable groups (e.g., number of people being housed, which vulnerable groups were being helped). Finally, with reference to the RHI and the RHCF, CMHC did not know whether priority vulnerable groups who were intended to benefit from approved projects were actually housed once projects were completed—for example, if units created for people with disabilities were actually being occupied by this population. Indeed, in evaluations of both the NHCF and the RCFI conducted in 2021, one of three recommendations offered by the CMHC concerned its data collection strategy, and in particular, exploring the *potential* of collecting tenant information—either directly or through an arrangement with Statistics Canada—to better measure the contribution of the program in providing affordable units to target groups and NHS priority populations (Canada Mortgage and Housing Corporation 2021b, 48; 2021c, 37).

Overall, the above signals that a considerable data and information gap exists with regard to the impact of NHS investments in new affordable housing supply, on vulnerable groups in particular. Not only is the present lack of robust and accessible data a significant barrier to assessing the impact and effectiveness NHS programs on priority groups, but it also represents a failure on the part of the federal government to uphold its obligations as part of its commitment to progressively realize the right to housing in Canada.

Existing Data Sources and Outstanding Gaps

Several data sources do exist in Canada which enable researchers and governments to track aspects of housing access and adequacy over time—that is, separate from particular NHS programs and investments—including for subgroups of people, such as those priority groups outlined in the NHS. These include the Census, which is conducted every five years and can be used to make determinations of shelter-to-income costs and core housing need, and can also support the creation of indicators that address various aspects of housing adequacy, and the Canada Housing Survey, which is conducted every two years (until 2028) and provides information on home, neighbourhood, and community satisfaction; needs and housing conditions of households living in social and affordable housing; and other measures of well-being linked to housing. These sources could be enhanced and expanded to better meet long-term tracking needs regarding the progressive realization of the right to adequate housing, including through an engagement with principles of data equity.

Additional information is available through the Homeless Individuals and Families Information System, Point-In-Time (PiT) Counts, By-Name lists of people experiencing homelessness, shelter use and capacity data, and social housing waitlists, and information in these sources is often disaggregated by some characteristics. PiT counts, which are conducted in a number of Canadian communities to determine the homeless population at a given point during the year. At present, roughly 30 communities in Canada also have By-name list which support tracking of the extent and nature of chronic homelessness. These data sources and the research and practice networks through which they are maintained constitute an excellent starting point for building out data collection and stewardship efforts related to NHS investments.

2. The NHS and Priority Groups

Progressive realization obligations require that governments give particular consideration to those who face the greatest barriers in exercising this right, including by tailoring housing initiatives to vulnerable and disadvantaged groups, rather than merely serving the general population through universal policies and funding mechanisms. Accordingly, the Government of Canada has named “housing for those in the greatest need” as one of six priority areas for action as part of the NHS, and has further identified 11 target sub-populations of vulnerable people in Canada whose needs will be addressed through the strategy. Several of the new supply programs highlighted above, particularly the NHCF and RHI, have specific objectives concerning units for priority populations. Understanding these priority groups, the current state of housing need among them, the barriers they face, and across which dimensions of housing adequacy, is vital for developing data stewardship efforts that not only result in the best available information, but which can also support long-term objectives related to the housing of these populations. This section provides an overview of the 11 priority groups named in the NHS, linking the housing barriers experienced by these groups to the notion of adequacy in a way that provides important contextual information within which to ground data and information stewardship efforts.

2.1. Vulnerable Groups and the NHS

The 11 priority groups identified in the NHS are women and children feeling domestic violence; seniors; young adults; Indigenous peoples; people with disabilities; people dealing with mental health and addiction issues; veterans; the 2SLGBTQIA+ community; racialized groups; recent immigrants, especially refugees; and people experiencing homelessness (Canada Mortgage and Housing Corporation 2023). However, this list reflects several notable omissions, among them migrants and persons with

precarious immigration status, persons who have been in contact with the justice system, persons with disabilities who require not only housing but complementary support services in order to live independently, lone women, lone caregivers, and rural and remote communities housing (Biss and Raza 2021, 95). As a result, those engaged in data stewardship efforts might choose to include questions which also assess the extent to which these groups are served by NHS new supply investments, despite their absence in the NHS itself.

In 2021-22, the Federal Housing Advocate partnered with Statistics Canada to release a series of 24 fact sheets on housing experiences in Canada for various population groups (see Statistics Canada 2021), including the majority of the 11 priority groups listed in the NHS. These fact sheets draw on data collected through the Canadian Housing Survey and the Census of Population and present key information on tenure status, shelter costs, housing affordability, housing suitability, condition of dwelling, core housing need, dwelling satisfaction, neighbourhood satisfaction, and household living arrangements. As these fact sheets provide a good picture of the extent of housing need experienced by many of the priority populations listed in the NHS, we—rather than reproducing this information here—suggest interested readers consider this resource directly.

While the Housing Experience fact sheets fill a longstanding informational gap regarding housing outcomes for diverse groups of people in Canada, they do not provide a contextual grounding for the need and experiences they outline, nor do they make mention of the myriad and intersecting systemic factors which shape experiences of housing and homelessness for diverse people in Canada. Indeed, the decision to discuss housing needs (and priorities) in terms of target vulnerable groups, rather than in relation to the systemic and structural factors which produce contexts of vulnerability and need, is antithetical to best practices in intersectionality. In particular, this type of approach risks overlooking the considerable overlaps among target groups, and the extent to which housing precarity is often experienced by those who are situated at the intersection of several aspects of vulnerability. Ignoring this reality means that housing policies which target one type of vulnerability—disability, for example—may exclude individuals within that target group who experience overlapping barriers and have corresponding needs (e.g., disabled Indigenous women in rural areas). In reality, housing need is a function of an individual's multi-dimensional experience and interactions within and across systems of privilege and oppression, access and exclusion, and empowerment and harm. A more useful way of discussing housing access in future strategies, then, might be to conceptualize need in terms of interlocking structural pathologies, systemic oppression, and the barriers they create for people of diverse identities. For example, one might ask how realities of settler colonialism, dispossession, inter-generational trauma, misogyny, and ableism interact to not only undermine housing access, but also produce particular conditions of housing adequacy, for Indigenous women living with a disability.

Understanding how systems of oppression and structural factors shape access to housing and elements of housing adequacy—particularly for vulnerable groups—is integral to assessing progressive realization and provides a grounding for equity-based data stewardship efforts to support such assessments. Frameworks which link data and information to these factors make it possible to take reporting and evaluation beyond a basic counting exercise that mainly serves those designing housing policy and administering investments, to advance a holistic approach that centres people and empowers communities to understand and communicate outstanding barriers and advocate for change. In Table 3, we outline some of the core systemic realities that create barriers to housing for diverse people, demonstrating how they can influence housing access and bringing in examples that pertain to particular priority populations and the domains of adequacy.⁹ Reflecting on housing barriers in this way shifts the burden of responsibility for

⁹ Housing adequacy should be conceptualized as living in security, peace and dignity, and in housing that fulfils a suite of conditions, not simply as the presence of a roof over one's head. Given diversity of identity, experience, and need, the basic conditions that must be satisfied in order for housing to be deemed adequate take on various permutations and combinations across space and time, and given the individual in question.

housing gaps away from the individual and onto the system, accounts for the myriad ways in which housing access is mediated by power and privilege, and illuminates the extent to which housing barriers are social and political constructions. It also enables a consideration of how people situated at the intersection of different social locations may experience particular types and aspects of housing need, rooted in and driven by overlapping structural pathologies and systems of exclusion and oppression.

Table 3: Systemic Factors Influencing Access to Adequate Housing

Systemic Factor	Connection to Access and Adequacy Barriers
Exclusive and Inaccessible Spaces	The way in which “standard” housing is constructed, as well as the typical features and services it offers, are not accommodative of many of the ways in which people live across the spectrum of dis/ability and at different points throughout the life course. Though building codes are being adjusted to incorporate better standards regarding universal and accessible design, among other concepts, many people in Canada—particularly renters and those with lower incomes, who may not have the capacity to construct accessible spaces or make adjustments to remove barriers—are forced to accept housing conditions that do not meet standards of adequacy, or end up abandoning independent living situations (Alzheimer Society of Canada et al. 2017; Weeks and Kristal 2010). The exclusivity of the built environment and a failure to consistently integrate accessibility in neighbourhood planning mean that core adequacy conditions—proximity to public transit, safe navigability of the surrounding neighbourhood using a number of modalities (e.g., walking, rolling), adequate common spaces and visitability, the presence of services nearby, and others (My Home My Community 2019)—are difficult to realize, particularly in rental markets.
Colonialism and Dispossession	Ongoing settler colonialism and racism have resulted in displacement and dispossession for First Nations, Inuit, and Metis people, which can be understood as homelessness along 12 dimensions: (1) historic displacement; (2) contemporary geographic separation; (3) spiritual disconnection; (4) mental disruption and imbalance; (5) cultural disintegration and loss; (6) overcrowding; (7) relocation and mobility; (8) going home; (9) nowhere to go; (10) escaping or evading harm; (11) emergency crisis; (12) climatic refugee (Thistle 2017). These dimensions are further associated with a number of access and adequacy barriers—for example, the forced location of Indigenous populations to reserves and settlements, alongside a subsequent failure by successive governments to provide adequate housing, and invest in services and infrastructure to ensure habitability, inclusion, and safety (House of Commons Standing Committee on Human Resources, Skills & Social Development, and the Status of Persons with Disabilities 2021, 31). Displacement to urban centres, often as a result of these conditions, also produces dynamics of exclusion, whereby individuals experience isolation and a disconnection from community and culture, and often no improvement in living conditions. Further, interconnected forms of structural violence long present within the housing market, child protective services, and homeless-serving system create substantial barriers for Indigenous women in urban settings, particularly in accessing housing security and healing from trauma (Rodrigues et al. 2020).

Gender-Based Violence	Violence is a major contributor to housing insecurity and homelessness among women and children in Canada, and intimate partner violence in particular undermines access to safe housing for women and children, both if they remain in abusive contexts, and as they seek to escape it (Maki 2017). In the presence of violence, safety and access to services and supports take on particular salience in definitions of adequacy. However, in fleeing IPV, many women and their children face compounding systems failures across social assistance, child welfare, social housing, and other public systems, which contributes to intergenerational cycles of housing instability, marginalization, and violence (Schwan et al. 2020). This structural oppression is compounded by other forms of discrimination (e.g., racism, ableism) and contextual factors, and thus takes on different expressions for women who belong to different communities. For example, in rural and northern settings IPV risk is more prevalent as a result of several overlapping forces, including a dearth of services and supports, isolation as a result of limited mobility, public visibility, patriarchal value systems, and greater presence of firearms (Jeffrey et al. 2019).
Homophobia and Transphobia	2SLGBTQIA+ communities, and transgender individuals in particular, face considerable discrimination in rental markets, and report being either denied approval for or evicted from housing on the basis of their identity (Lauster and Easterbrook 2011; McDowell 2021). Evidence from British Columbia highlights that queer women report difficulty in accessing housing with female partners (including in comparison to when they sought housing with male partners), to the extent that many took on the strategy of hiding their sexuality and relationships in order to access and maintain housing, including in shelters and supportive housing (Lyons et al. 2019). Further, transgender persons, including youth, experience considerable structural barriers in accessing shelters due to identity-based discrimination. For example, research indicates that one in three transgender youth will be rejected by a shelter on account of their gender identity and/or expression (Abramovich 2014).
Racism	Canadian studies have found that prospective tenants who belong to Indigenous (Cohen and Corrado 2004; Malley 2016; Johnson 2019; Native Women's Association of Canada 2019), Black (Mensah and Williams 2013; Teixeira 2008; Vincent 2019), and immigrant (Centre for Equality Rights in Accommodation 2013) communities face considerable discrimination and stigmatization in rental markets on the basis of race, which can make it difficult to find and keep adequate housing. This includes landlords openly asking about drinking and drug use, cultural and culinary practices, as well as number of children and extended family members in the household, during the search process.

3. The State of Knowledge on Data Equity

Data can serve as a powerful tool for systemic change. When gathered and analyzed in a way that foregrounds principles of justice and an ethic of care, data can uncover inequalities, be used to craft policies that respond to diverse needs and fill identified gaps, provide for effective monitoring and evaluation of programs and progress towards strategic goals, and support the transformation of systems and structures towards anti-racist, de/anti-colonial, and inclusive ends. The collection of disaggregated data in particular can reveal patterns or experiences that are obscured in aggregate data—including the nuances of inequalities and differences that exist among sub-groups—helping to hold governments and institutions to account for obligations to diverse groups of rights claimants (BC OHRC 2020), including in the context of the right to adequate housing. Indeed—and as is currently the case with NHS new supply programs—without information about the various sub-populations reached through housing policies (and their resulting experiences), it is difficult to discern where programmatic gaps exist and where tweaks are

warranted; to gauge how governments are faring in addressing the needs of priority groups through broad and targeted policies; and ultimately, to assess whether housing strategies are succeeding in realizing the right to housing in a general sense.

Despite the central role that data and information can play in advancing progressive agendas, the history of data collection is a fraught one, reminding us that without proper attention and care, data stewardship efforts can simply reinforce inequalities. Specifically, data have long been used to stigmatize disadvantaged groups and to advance harmful, discriminatory, and pathologizing narratives which reinforce power structures and systems of control. Data collection efforts have also supported surveillance, dispossession, and other forms of violence, to say nothing of the often harmful effects of the research carried out as a result of access to this information. In other cases, certain vulnerable populations, such as persons with disabilities, have been rendered invisible through data collection efforts that fail to properly account for their experiences, or which create barriers to participation (United Nations Children's Fund 2020). Given growing attention to the necessity of better data and information in the context of the NHS, it is vital that the collection efforts that result are undertaken in a way that not only foregrounds equity, but which also explicitly aims to reverse or overcome these histories, dynamics, and failures.

The state of knowledge concerning equitable data collection and management is growing, particularly in research fields such as health. In this section, we draw on UN guidance and other literature from international bodies, as well as frameworks developed by a range of organizations in the Canadian and American contexts, to review the state of knowledge on equitable data stewardship. Specifically, we draw out key concepts and principles, note barriers to data equity, and highlight a number of wise and promising practices. This work will form the basis for the equity-based data collection framework presented in Section 4.

3.1. Defining Data Equity

One of the best expressions of the spirit and purpose of data equity can be found in the guidance on disaggregated data collection standards issued by British Columbia's Office of the Human Rights Commissioner, which states that "disaggregated data is merely a tool (to be utilized depending on the context and aim) and [...] must be accompanied by a process that supports the purpose of reducing systemic racism and oppression and achieving equality" (BC OHRC 2020). That is, while gathering disaggregated data can provide authorities and communities with statistics and information to analyze the impacts, impartiality, and fairness of government policy and programs, particularly on marginalized populations (White House Equitable Data Working Group 2022), in the absence of an equitable process and governance mechanisms, the collection of such data cannot be relied upon to advance equity goals. In accordance with this distinction, the BCOHRC guidance discards a primary focus on data collection, and elevates notions of purpose and process, tied to systemic change. This perspective signals both that disaggregated data is but one element of equitable data stewardship—and indeed ancillary to other aspects—and that, in the absence of a transformative purpose and a process grounded in relationship and care, the collection of data and information is likely to reproduce the very inequalities it aims to address. This idea is borne out in the vast body of evidence that suggests that the mere presence of data indicating service gaps, inequalities, and systemic oppression for diverse groups does nothing to guarantee policy reform or material change. As Rinaldo Walcott (2021, 81)—writing on the recent push for race-based data—underscores, "what we have witnessed is that research and the link to better policy is not self-evident and that researchers and the populations researched can constitute a range of unequal relations in which the benefits of the research flow to the most advantaged in the relationship." Put simply, both data collection and research will remain extractive and exploitative if efforts are not first made to adjust both the context within which they operate, as well as the ends towards which they are put.

At its core, then, equitable stewardship is about disrupting the unequal power relations, dynamics, and approaches that undergird data collection, management, and the research and evaluation process more broadly. Rooted in respectful relationships and imbued with critical and transformational intent, equitable data stewardship involves a cross-cutting commitment and the adoption of attendant processes to advance equity, mitigate harm, and eliminate systemic injustice across all phases of the data and research cycle (BC OHRC 2020; Black Health Equity Working Group 2021; Nelson et al. 2020). In practice, this involves power sharing, relationship building, and the explicit engagement of community and marginalized voices in planning, agenda-setting, collection, analysis, and management, including by adopting inclusive participatory governance approaches for data access and use; advocating for research methods, including qualitative, participatory, and intersectional analysis, to better capture and provide context for diverse experiences; and reporting data results in a way that communicates context, is tailored for different audiences, and which is easily linked to positive action (Nelson et al. 2020; Nelson and Zanti 2020). It is this set of commitments that mitigates against the potential harms of disaggregated data collection efforts. Further, equitable data stewardship requires frameworks and processes which integrate accommodation and trauma-informed strategies, particularly during the collection stage, to ensure that all people are able to participate free from barriers and harm.

3.2. Data Equity Guidelines and Frameworks

Several research institutes, human rights groups, and non-governmental organizations have outlined principle-driven frameworks for advancing equitable data stewardship, the elements of which can be adapted for use as part of data collection and management efforts specific to NHS programs and priority groups. Table 4 summarizes eight of these approaches, which span general equity-driven, human rights-based, anti-racist and decolonial, and disability-inclusive frameworks and toolkits. Several core principles are discernable across these frameworks, indicating a set of important elements that can be used to structure equitable data stewardship efforts. These include the necessity of engaging with and finding ways to integrate the participation of impacted communities across all aspects of the data stewardship process, particularly in setting collection processes and analyzing results; ensuring that those collecting and analyzing data have appropriate training in inclusivity, accessibility, and cultural sensitivity to minimize harm; establishing clear accountability in terms of the purpose and use of the data; committing to transparency regarding process, objectives, collection methods, and approaches for data management; and ensuring the protection of data through security and confidentiality measures.

3.3. Additional Concepts

In addition to the above principles, concepts such as intersectionality and data sovereignty are integral to data equity efforts—and many of the above frameworks draw on and integrate these ideas. By embedding such understandings throughout the data stewardship process—from planning through to analysis and destruction of data—those who are leading data efforts can ensure that the information gathered is used to improve outcomes for vulnerable groups, while avoiding unintended consequences and the perpetuation of harm. These concepts are particularly helpful in ensuring engagement with assumptions, potential harms, and other power-based implications of data stewardship during the planning process; in thinking about and designing governance and management approaches; and in shaping collection strategies (including mechanisms and people involved), wording questions, and contextualizing information and findings to inform better analyses.

Table 4: Comparison of Best Practices in Equitable Data Stewardship

Framework	Focus	Principles
A Human Rights-Based Approach to Data (UN OHCHR)	All rights claimants	Six principles + attendant practices to inform data collection in support of human rights goals: 1. Participation of relevant groups across data collection exercises. 2. Collection of disaggregated data , including on characteristics identified in IHR law. 3. Populations should be self-defining , and must have option to disclose/withhold information. 4. Transparency : Public must have open access to data, clear information on design & methods. 5. Privacy : Confidentiality of individual responses and personal information must be maintained. 6. Accountability : Data should be used to hold states and other actors to account on human rights issues.
Count me in! Collecting human rights-based data (Ontario Human Rights Commission 2009a)	All protected parties under Ontario <i>Human Rights Code</i>	Six steps for organizations collecting and analyzing data that identifies people on Code grounds: 1. Identify issues and/or opportunities for collecting data, including context. 2. Select issues and/or opportunities and set goals. 3. Plan: Collected about whom, in what form (quant., qual.), how (e.g., surveys, interviews.), for how long? 4. Collect data. 5. Analyze and interpret data. 6. Act on results to address barriers: set goals, obtain community input, measure and report progress
Collecting data in a Code-consistent way (Ontario Human Rights Commission 2009b)	All protected parties under Ontario <i>Human Rights Code</i>	Seven guidelines to ensure data involving Code and non-Code groups is collected and used appropriately: 1. Collect data for a Code-consistent purpose. 2. Inform public and collection populations regarding reason information is being collected, potential uses, collection approach, steps to be taken to protect privacy, and progress reached in achieving objectives. 3. Consult affected communities about the need for data collection and appropriate methodology. 4. Use the least intrusive means. 5. Anonymity: Take measures to ensure that privacy and confidentiality are maintained. 6. Distinguish between collection, use and disclosure: there should be a rational and objective connection between the nature of information being collected and intended use. 7. Information and privacy: must comply with freedom of information and privacy protection legislation.
Urban Institute: Principle-Aligned Practices for the Data Life Cycle (Gaddy and Scott 2020)	All affected communities, groups of people	Approach for integrating the Belmont principles throughout the four stages of the data life cycle: Acquisition : include community in design; consider sensitivity; minimize amount of personally identifiable information; consider re-identification risk; avoid undue burden; incorporate informed consent Processing and Analysis : incorporate community interpretation of data; be transparent about data limits Dissemination : account for how publication could reinforce inequities or close disparities; share data to reduce duplication; return data and results to community in a form they can use Disposition : empower individuals to order data destruction; be transparent about long-term data plans
Producing Disability Inclusive Data (United Nations Children's Fund 2020)	People with disabilities	Six steps for breaking the cycle of invisibility for persons with disabilities: 1. Developing inclusive methodologies and instruments 2. Implementing inclusive study designs and methods of data collection 3. Representing the experience of persons with disabilities in data analyses and results 4. Ensuring inclusive reporting and dissemination 5. Promoting discussion, learning, and reflection about disability 6. Making evidence available to guide inclusion strategies and policy development.

EGAP: Engagement, Governance, Access, and Protection (Black Health Equity Working Group 2021)	Black communities	Four-dimensional framework for health data collected from Black communities in Ontario: 1. Engagement: Genuine, cyclical, accessible consultation with communities regarding data collection, management, analysis, and use. 2. Governance: Community decision-making about engagement processes and data collection, management, analysis, and use, achieved through the establishment of Community Governance Tables. 3. Access: The right of communities to access their collective data and to determine who else can access it, along with the capacity building required to enable this right. 4. Protection: The safeguarding of all individual rights and types of data, including identifiable, de-identified, and anonymized data.
The Grandmother Perspective: A Framework for Disaggregated Data Collection (BC OHRC 2020)	Race-based, Indigenous, other data	Three-pronged framework for the collection of data: 1. Purpose (equity and the elimination of systemic racism and oppression): Define the project purpose in relation to a specific equity goal, while ensuring focus is on systems failure, aligning purpose throughout data cycle, describing why data is needed, defining targets in line with purpose and expected structural changes, and developing appropriate accountability mechanisms. Embed social construction of demographic groups throughout, including by considering influence on research questions and analysis. 2. Process (respectful relationship grounded in community data governance): Identify impacted communities and establish working groups, empowering members to participate by removing barriers and extending decision making authority. Establish evaluation throughout process. 3. Tool (disaggregated data collection, if it advances purpose): Validate purpose and define research questions relevant to community. Identify whether other data sources (e.g., qualitative data) would generate better analysis and avoid perpetuating harm. Determine specific questions in relation to demographic categories appropriate in context of project. Think about timing of surveys. Apply informed consent protocol. Use Safe Data, Safe Settings, Safe People principles for data management.
AISP, UPenn: A Toolkit for Centering Racial Equity throughout Data Integration (Nelson et al. 2020)	Race-based data	Positive and problematic practices for centering racial equity across six stages of the data life cycle: 1. Planning: Identifying stakeholders; convening work groups; articulating purpose; understanding racial, social, and historical context; creating ethical guidelines for use; developing project plan. 2. Data collection: Defining which data should be collected, reused; developing shared data collection agenda with communities, agencies; including qualitative stories; understanding why people opt out. 3. Data access: Ensure community confirms value of open data; clear release schedule; adhere to best practices in access, de-identification; free/low cost for requests; clear information on data unavailability. 4. Use of algorithms and statistical tools: attention to how principles of responsibility, explainability, accuracy, auditability, and fairness will be operationalized in algorithm use. 5. Data analysis: Using participatory research in interpretation; engaging various experts (agencies, researchers, communities); correlating place to outcomes; employing mixed methods; disaggregating data; using intersectional analysis. 6. Reporting and dissemination: Communicating findings using range of tools adapted to audiences; reporting data in actionable way, in connection with policy options; acknowledging structural dynamics embedded in data; public access through reports, dashboards, etc.; documenting process clearly

Intersectionality

Though it is often associated with critical feminist analysis, intersectionality is an analytical approach that abandons single-axis considerations of gender and invites a focus on the ways in which multiple and interlocking systems of power interact to produce privilege and oppression for diverse people on the basis of their social location, as well as structure the world through their influence on norms, narratives, practices, and institutions (Collins 2015; McCall 2005). The concept is most often associated with the work of Kimberlé Crenshaw (1989, 1991) who, drawing on the image of intersecting roads, demonstrated how the interaction and compounding effects of multiple social locations or categories, such as gender, race, ability, and class, produce multifaceted forms of oppression distinct from that which is experienced as a result of belonging to one of these groups. For Crenshaw, demarginalizing oppressed groups within systems and policies required the development of integrative analytical frameworks capable of engaging with the notion of the intersection.

As summarized in Cameron and Tedds (2023), intersectionality hinges on four core principles concerning identity, power, and the social world: (1) identity is multi-faceted and socially constructed,¹⁰ and the categories and/or systems that shape identity must be considered together—as co-constituting—rather than independently; (2) the social world and our institutions, such as government and the housing system, are both sites of power and control, as well as the places in which identity production and experience play out; (3) systems of power and oppression, such as colonialism and racism, underpin and are reproduced at the individual, social, and institutional levels; and (4) the processes and relations power operates through are interlocking and reinforce one another. The diagram in Figure 1, taken from Cameron and Tedds (2023), depicts an intersectionality-driven conceptualization of individual identity formation and experience, as they unfold through relations and interactions across societal and institutional contexts. Each circle corresponds with the level of analysis—intersecting social locations, sites and institutions, and power structures/systems of oppression—that interact to shape an individual’s identity and experience of the world. The arrows linking each layer indicate the interconnectedness of these processes and systems: that is, that each influences the others, and that they cannot be considered in isolation.

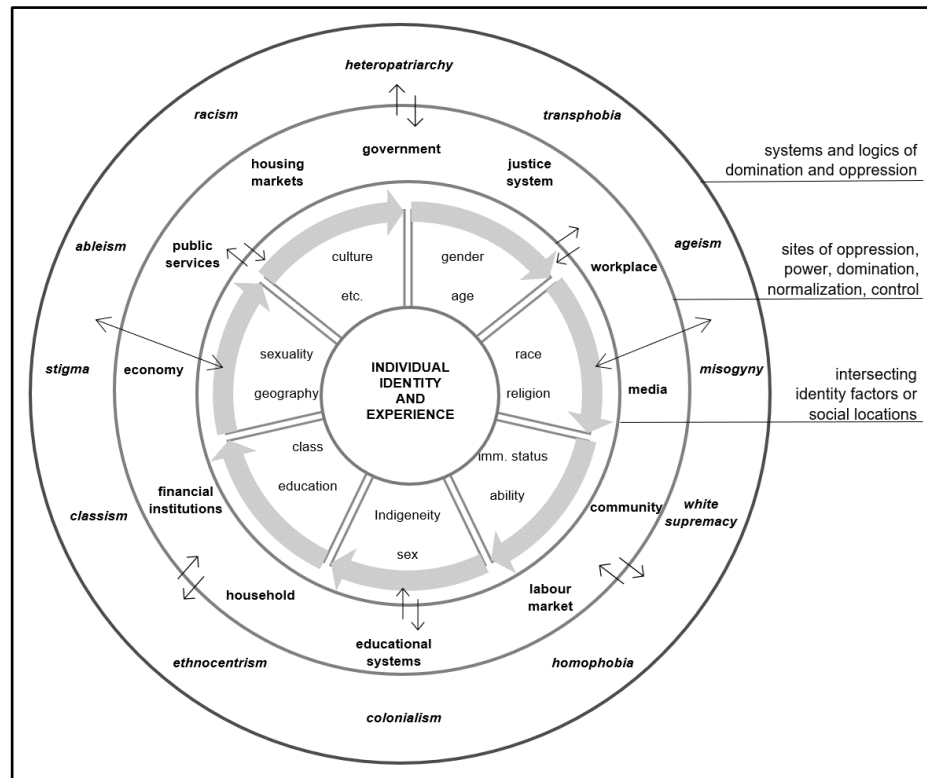
Best practice analysis in the context of data equity indicates that an intersectional lens should be used to inform data collection efforts, enabling those tasked with stewardship to foreground notions of intersecting oppression, social construction of identity, power, and systemic context when designing methods, crafting survey approaches and questions, performing collection activities, and undertaking analysis. For example, BCOHRC’s data stewardship framework requires that intersectionality is embedded in all elements of the data process, including in determining categories for disaggregated data collection, designing survey questions, and analyzing data (BC OHRC 2020, 47). Here, intersectionality is vital to understanding how systemic oppression comes to bear on differently situated populations, and to ensuring context is provided for the outcomes measured and reported on. The AISP Toolkit for Centering Racial Equity throughout Data Integration (Nelson et al. 2020) and the EGAP Framework (Black Health Equity Working Group 2021) also note the value of bringing an intersectional approach to data analysis.

Further, taking an intersectional approach across data stewardship efforts will enable CMHC and other involved groups to align their work with Canada’s GBA+ framework, and as a result, maintain commitments set out in the NHS regarding the application of GBA+ throughout the course of NHS efforts, including monitoring and implementation. GBA+ is the federal government’s primary approach to

¹⁰ The social construction of identity (or categories) refers to the fact that the various aspects or social locations that compose our identities—that is, the categories that are measured when collecting disaggregated data—are not (for the most part) biologically defined, but socially, politically, and historically constructed (BC OHRC 2020). As a result, it can be helpful to think about identity categories as directly produced by the world around us—or about how policies or systems a “gendering, heteronorming, classing, (dis)abling and racializing in their effects” (Bacchi and Eveline 2010, 120).

addressing inequities within the policy space (Christoffersen and Hankivsky 2021), and serves a way for those developing policies and programs to assess the potential impacts of legislation, policy, and initiatives on diverse groups of people using intersectional principles.

Figure 1: Visual conceptualization of intersectionality



Source: Cameron and Tedds (2023)

Data Sovereignty

Data sovereignty refers to communities having control over their collective data and is most often discussed in relation to Indigenous rights—though some groups, such as the Black Health Equity Working Group, are advancing similar work in relation to Black communities in Canada. Specifically, Indigenous data sovereignty reflects the inherent right of Indigenous peoples to exercise authority over their data and information—that is, data about them, as well as their lands, territories, and ways of life—through both stewardship and processes of control, such as collection, storage, analysis, use, and reuse (Carroll et al. 2020; The First Nations Information Governance Centre 2019). In recent years, as global efforts towards Indigenous self-determination have advanced, and as big data, open data, and data reuse have gained prominence, Indigenous data sovereignty has emerged as an important avenue by which Indigenous peoples are reclaiming identity and knowledges (Carroll et al. 2020).

In Canada, OCAP (Ownership; Control, Access, and Possession) principles have been developed by First Nations communities as a set of standards for the collection, protection, use, and sharing of First Nations' data and information (First Nations Information Governance Centre n.d.)—what Schnarch (2004) refers to as self-determination applied to research. Specifically, the OCAP principles refer to:

- Ownership: That a community or group owns information collectively in the same way that an individual owns their person information—as distinct from stewardship (which is a mechanism

through which ownership may be asserted, and involves caretaking of data or information by an institution accountable to the group);

- **Control:** That First Nations Peoples and their communities are within their rights in desiring control over all aspects of research and data management processes which impact them. This includes all aspects of a research project, and to the control of resources, review processes, conceptual frameworks, and data management;
- **Access:** First Nations Peoples must have access to information and data about themselves, regardless of where it is held, and also have the right to manage and make decisions regarding access to their collective information; and
- **Possession:** The right to possess data, as a mechanism by which ownership can be asserted and protected. (First Nations Information Governance Centre n.d.; Schnarch 2004).

Building on OCAP, the Global Indigenous Data Alliance has published a global set of principles—the CARE Principles for Indigenous Data Governance (Collective Benefit; Authority to Control; Responsibility; and Ethics)—which position data governance within Indigenous cultures and knowledge systems, and for the benefit of Indigenous Peoples, but apply to research, government, and other institutional data settings (Carroll et al. 2020). CARE denotes that:

- Data ecosystems should be designed in ways that enable Indigenous peoples to derive benefit from the data, spanning inclusive development, improved governance, and equitable outcomes;
- Indigenous Peoples’ rights, interests in, and control over their data must be recognized and empowered, including through cultural governance protocols for stewardship and data access;
- Those working with Indigenous data have a responsibility to reveal how data are used to directly support self-determination and collective benefit of Indigenous peoples, and to invest resources in generating data grounded in Indigenous languages, worldviews, and lived experiences; and
- The rights and well-being of Indigenous Peoples are the primary concern at all stages of the data life cycle and across elements of the data ecosystem, and to this end, data should minimize harm and maximize benefit, support justice, and take into account potential for future use and harm (Research Data Alliance International Indigenous Data Sovereignty Interest Group 2019).

In recent years, additional approaches to advancing data sovereignty—which draw on OCAP and CARE—have emerged in Canadian communities as a result of local initiatives. Among them are the Black Health Equity Working Group’s (2021) EGAP framework (Table 3). Further, the Canadian Institute for Health Information (CIHI) is one organization in the Canadian context that has taken an approach to the governance of First Nations, Inuit and Metis data that foregrounds a commitment to data sovereignty principles. This involves developing foundational capacity within CIHI through cultural safety, humility, and responsiveness training; by building relationships and partnerships at all levels with Indigenous groups; by aligning policies, practices, and procedures with Indigenous data sovereignty principles (including by signing information management agreements with communities, allowing communities to access data at any time and to determine whether and how their data is shared, and limiting third-party requests); and by supporting actionable analysis and capacity building in collaboration with Indigenous communities (Canadian Institute for Health Information 2020a). Together, these approaches must be considered within the context of data collection and broader stewardship efforts that involve such populations, even if principles are adopted within frameworks without full extension of sovereignty.

3.4. Overcoming Barriers to Data Equity

Though data can have a significant impact on policy development and can produce transformative change for vulnerable populations as a result, several barriers exist to achieving data equity linked with the perpetuation of harm. The BCOHRC (2021) summarizes harms associated with disaggregated data collection, noting that overall, data can reinforce stigma and systemic oppression against marginalized

communities, present deficit narratives and reinforce the idea that such groups are to blame for their own marginalization, be used as a form of surveillance and control, and pose a risk regarding re-identification, particularly given rapidly evolving technology. In the Canadian context, the collection of information has been used as a tool of colonization—as a way of pathologizing, controlling, and taking action against Indigenous peoples—and to advance devastating and racist policies, such as the forced removal of Indigenous children from their family homes, whether through residential schools, the Sixties Scoop, or contemporary child welfare systems (McBride 2018, 6). Moreover, race-based data collection and analysis, in addition to advancing narratives that blame Black communities for structural problems, has historically advanced harmful and scientifically-baseless theories which draw links between inequities and genetic disposition (Black Health Equity Working Group 2021). Other Canadian examples of harmful data practice include extensive record keeping on, as well as exposure and removal of, LGBTQ2S+ members of the federal public service until the 1990s, and the documentation, tracking, and internment of Japanese communities in British Columbia during the Second World War (BC OHRC 2020). Further, many members of marginalized groups resent the frequency with which they are asked to perform the role of study subject—particularly given a lack of say in data interpretation and use, and limited attendant improvements in their material realities (McBride 2018; Schnarch 2004; Walcott 2021). The risk of harm undergirds the entirety of the data stewardship process, including the intent driving the collection, the design of questions and collection mechanisms, the collection process itself, the analysis of data and information gathered through the process, and the publication and dissemination of findings.

Weak Social Category Design

The way in which data collection is designed can undermine the extent to which the collected data and information is both representative of reality, as well as useful in making determinations about outcomes and future policy directions. For example, social categories can be poorly defined, and both result in under or misreporting of information associated with certain identity categories, as well as contribute to reinforcing harm and erasure of identity and experience. In the Canadian context, common issues with social categories used in surveys include the imposition of pan-Indigenous demographic frameworks that do not represent the spectrum of Indigenous diversity; only including binary gender markers, or using a category of ‘other’ to represent identities which a binary does not account for; assuming broad racial categories such as “Black” are inclusive of all people one might class in that way; and failing to represent experiences across the spectrum of disability (BC OHRC 2020). A primary strategy for overcoming these issues is involving target groups in the development of social categories, markers, and other survey questions that concern them. By designing categories in line with the self-identification of community members, those crafting surveys can ensure language around social categories reflects that used by the community. In addition, those identifying social categories should not think in terms of binaries (e.g., able/disabled; woman/man), but instead bring in concepts of identity as existing along a continuum, as well as provide the option to self-report and/or select several markers under a particular category.

Data standards which incorporate the above (and other) best practices are emerging within key research and statistical agencies in Canada. In the context of race-based and Indigenous identity data collection and reporting, the CIHI (2020b) has refined ways of developing questions and response categories that foreground the social construction of identity, provide open-text response options for people to use if categories do not represent how they self-identify, and the option to collect more granular data, where relevant (e.g., specific sub-populations, such as African Nova Scotians or particular Indigenous communities or Nations. Further, one way in which Statistics Canada (2020) is moving away from the binary in the context of sex and gender is to implement a two-step process for defining variables and questions that generate categories reflective off a mix of information on both category types, depending on response. Specifically, this involves first collecting information on sex at birth and then current gender identity within a spectrum. Such a process was recently implemented as part of the 2021 Census. Here, gender can change over a repeated sample, and this must be accounted for over the course of the work.

Barriers to Participation

Even if the content of surveys and other data collection tools has been crafted with equity in mind, a failure to address participation barriers can undermine the success of the data collection process, including through low uptake and response rates. Barriers to participation include those associated with accessibility (e.g., language, impairments, technology), lack of comfort in sharing personal information with institutions, feelings of shame, and fear of facing stigmatization or harm as a result of participation. In some cases, such as with women fleeing violent contexts, having to provide information about one's experience may trigger trauma; others, such as sex workers and Indigenous people, may be uneasy about the sharing personal details with government institutions, for fear of it being weaponized against them. Data collection processes must acknowledge these realities. A failure to identify and account for these barriers to participation is particularly important in the context of evaluating the NHS new supply programs, given the profiles of the 11 priority groups.

Strategies for removing barriers to participation can draw on a range of measures, depending on the communities involved, from ensuring those administering surveys have undergone sensitivity training and are informed of core concepts, such as anti-racism, intersectionality, and gender and disability justice, and/or have established relationships of trust with the subject, to ensuring that proper investments are made in technologies, supports, and remuneration to enable full participation in a manner that is dignified. Data stewards can also create a safe and comfortable space for people to share personal information about their experiences by grounding the stewardship process in relationship and by being clear about the purpose and intent of the collection process, as well as each component part.

Shortcomings in Analysis and Reporting

Finally, analysis must avoid perpetuating harmful deficit narratives, which result when particular identity groups are linked with poor outcomes in the absence of context, as well as a tendency to treat target populations as monoliths. Those leading data stewardship efforts should ensure that those responsible for data and information analysis and reporting have knowledge of, or are advised by those with expertise in, critical methods such as intersectionality, as well as familiarity with critical race theory, queer theory, disability studies, and other relevant areas (BC OHRC 2020, 78). Further, interpretation of results should not be done without an understanding of structural and historical context, and the communication of findings should always foreground context and structural drivers of outcomes, rather than emphasize individual responsibility or deficiencies. Intersectionality is particularly important in the context of the NHS as there will be considerable overlap across the 11 priority groups—that is, many respondents will belong to more than one group—and researchers will need to understand how to account for and represent these complexities and overlaps in their analyses. One strategy for grasping context and intersections in analysis is by using mixed methods approaches, which enable researchers to supplement quantitative findings with more textured accounts of experience supplied through interviews and other forms of long-form, narrative-based reporting (Nelson et al. 2020, 29).

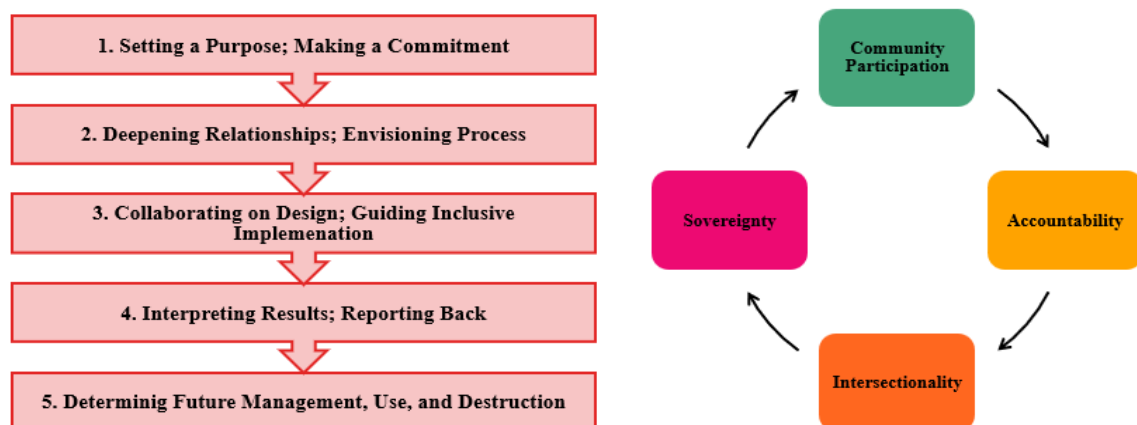
4. An Equity-Based Data Stewardship Framework

Drawing on the principles, promising practices, and intersectionality-informed understanding of housing access outlined above, we set out an Equity-Based Data Stewardship Framework for NHS new supply initiatives. Ultimately, the goal of this framework is to support the federal government, CMHC, and other relevant groups, such as Indigenous organizations, non-profits, housing providers, and community groups, to gather and steward data and information that can support assessments of the impact of NHS new supply initiatives, with a focus on the priority vulnerable groups mentioned in the strategy. Specifically, the data and information collected and analyzed through this approach should allow for answers to a two-pronged question related to Canada’s human rights obligations in the area of housing:

1. Who is accessing housing as a result of NHS new affordable housing supply programs—that is, who is behind the doors of NHS investments?
2. Are NHS investments in new supply supporting housing adequacy, including for those in the 11 priority groups? What are the experiences of people housed through these programs, what gaps and opportunities does this reveal, and how does this factor in to obligations under the NHSA?

The Equity-Based Data Stewardship framework is composed of a five-pronged process spanning purpose setting; relationship building and planning; data and information collection design and implementation; analysis and results sharing; and long-term management and use. Four attendant principles—community participation, accountability, intersectionality, and sovereignty—are set alongside this process and are intended to undergird all elements of the work and actions taken to advance it. These principles emphasize the importance of prioritizing relationship building and the involvement, empowerment, and capacity building of priority groups throughout the process; the requirement that all data stewardship activities and tools be guided by, in service of, and accountable to an overarching purpose, which is grounded in systemic change and fulfilment of rights; that stewardship activities—particularly collection and analysis—should be grounded in an intersectional approach, which foregrounds diverse representation, intersecting identities, context, the social construction of identity, and systemic (rather than individual) responsibility for housing outcomes; and the requirement to actively work towards the integration of data sovereignty principles where possible, appropriate, and desired by communities. Each of the five phases of the framework is detailed in the panels that follow, with particular attention to key aspects, potential barriers, and practice-based recommendations.

Figure 2: An Equity-Based Data Stewardship Framework: Process and Guiding Principles



Importantly, this framework constitutes a guideline or blueprint—a starting point—for data stewardship frameworks (or frameworks) that must be developed in direct partnership with affected groups, including those with lived experience of housing need spanning at least the 11 NHS priority groups. As a result, the spirit, content, and recommendations set out in this framework should only be replicated if in accordance with the principles, objectives, and needs of such groups.

1. Setting a Purpose; Making a Commitment

Overview: Equitable data stewardship must always begin with the establishment of a clear purpose or intent, tied to actionable plans and rooted in an overarching commitment to systemic change; without one, data efforts risk being extractive—in failing to move beyond collection and analysis, to action, for example—and can cause further harm to the communities about which the information is being collected (BC OHRC 2020; Black Health Equity Working Group 2021). This stage of the process involves setting that purpose, linking it to high-level commitments associated with systemic change, understanding the context within which the purpose is being pursued, and beginning to think critically about potential harms, community involvement, and opportunities to embed sovereignty principles.

Key Elements

1. Begin at the beginning:
 - Ask “Why are we doing this work? Why is it necessary? Will it advance human rights goals?”
 - Question the assumptions undergirding the work and how the work could produce harm.
 - Establish context spanning historical, political, and social elements, including with regard to the experiences of the 11 NHS priority groups.
2. Articulate a clear purpose for the work, linked to structural change:
 - Examples: “access to adequate housing for all,” or “removal of systemic barriers to housing adequacy for diverse groups.”
 - Connect the purpose to the need for disaggregated data and information.
 - Identify actionable commitments, including targets and indicators, in line with advancing the purpose, placing focus on the structural change that will result from the work.
3. Plan for participation:
 - Map, identify, and develop an engagement plan for members of priority groups, which will guide efforts to enable their participation across the data stewardship process.
4. Identify preliminary opportunities to advance data sovereignty efforts across process.
 - Assess how sovereignty principles might be embedded throughout work.
 - Consider engagement of key organizations and other stakeholders in Indigenous and other communities who have capacity and desire to participate.

Potential Barriers

- Upfront equity work has a tendency to be ignored, particularly in contexts where there is pressure and urgency surrounding outcome reporting—as is the case with the NHS.
- There may be hesitancy to think beyond current conceptualizations of goals, objectives, and accountabilities within the NHS.

Practice-Based Recommendations

- CMHC should consider reframing its approach to data collection, away from a focus on reporting on number of units and “who is behind the doors,” and towards questions and analysis of greater benefit to the populations on whom the data is being collected.
- CMHC should consult with the Federal Housing Advocate as part the process to identify potential harms associated with the planned data collection.
- CMHC should partner with the Office of the Federal Housing Advocate to map, identify, and engage key community partners who can facilitate the engagement of members of NHS priority groups across all elements of project and stewardship process, by NHS initiative if appropriate.

Accountability

Participation

Intersectionality

Sovereignty

2. Establishing Relationships; Envisioning Process

Overview: All guidance on equitable data collection and stewardship emphasizes the central importance of relationship building and community engagement to enable the involvement of groups of interest in all aspects of data stewardship. As a result, the second stage of the process places focus on establishing the relationships and networks of trust to ground data collection, analysis, reporting, and management. Importantly, this step is also used to adjust and affirm, through the participation of community representatives from all 11 NHS priority groups, the preliminary purpose, goals, and commitments set out in the first stage.

Key Elements

1. Establish Relationships as Mapped in Engagement Plan:
 - Begin by engaging a working group of trusted organizations (e.g., provincial non-profit housing associations; organizations which serve priority groups, the Office of the Federal Housing Advocate; the National Right to Housing Network; Canadian Centre for Housing Rights, etc.) to advise on further engagement of/outreach with community members.
 - Ensure accessible and authentic engagement with community members, including by removing barriers to participation through technology, supports, and remuneration to lay foundation for co-design of collection, analysis, and management, beginning with identified and trusted organizations.
 - Co-determine relevant participation opportunities for community members over course of process (e.g., community organizations or non-profits to support data mapping and survey administration; people with lived experience/in priority groups to provide feedback on question development, data analysis, etc.)
2. Work with engaged partners to review, adjust, and affirm commitments:
 - Examples: “access to adequate housing for all,” or “removal of systemic barriers to housing adequacy for diverse groups.”
 - Connect the purpose to the need for data and information.
 - Identify actionable commitments in line with advancing the high-level purpose.
3. Outline opportunities for and commitments to learning and capacity building related to all aspects of data stewardship, in line with sovereignty principles:
4. Publicize process, goals, and approach.
 - Make clear key terms, approaches, goals, and the overarching purpose.

Potential Barriers

- Coordination of partners across priority groups, geographic regions, and investment types could prove difficult.
- Establishing consensus regarding high-level commitments and goals may take time, skill, and resources given the range of partners and experiences reflected in 11 priority populations.

Practice-Based Recommendations

- CMHC should make public its intention to engage in data collection on the topic of populations reached through NHS investments in new supply, as well as clearly communicate the purpose and expected outcomes (including accountability for actionable plans) of its planned efforts, including by funding NHS funding initiative.
- CMHC could consider working with partners to adopt Community Governance Tables in each province or for each priority group, as advocated by the Black Health Equity Working Group (2021), as a mechanism for managing and remaining accountable for participation objectives.

Accountability	Participation	Intersectionality	Sovereignty
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3. Collaborating on Design; Guiding Inclusive Implementation

Overview: The design of data collection strategies must foreground consultation with community and integrate inclusive and trauma-informed approaches to reduce, within a degree of reasonability, harm reduction that can occur as a result of the collection of data and information. This phase focuses on taking the time to work with community organizations and target groups to craft an approach that minimizes harm, enables full participation, and collects only that data and information which is needed to fulfil the project purpose.

Key Elements

1. **Establish a clear link between project purpose and data collection needs and plans.**
2. **Determine data needs:**
 - Conduct data mapping in partnership with non-profit organizations and housing providers, to determine what data already exists by NHS initiative, and how efforts might be integrated within existing information collection mechanisms, such as intakes, or be added alongside current databases (social housing lists, etc.).
 - Collection goals: Establish, with community advice, what data and information should and should not be gathered.
3. **Collaborate with community on collection design:**
 - Identity-based questions: Work with community partners to establish demographic categories, spanning vulnerable groups in NHS. This should be done in line with best practices, such as those set out in the guidance developed by the Canadian Institute for Health Information (2020b). As examples, this includes allowing for the selection of multiple identity factors related to race, as well as the option to report identity on own terms.
 - Housing adequacy questions: Draft range of questions spanning dimensions of housing adequacy, drawing on knowledge of intersectional insights and structural context. Questions should strike at the root of inequalities, and incorporate diverse notions of accepted concepts.
 - Data collection mechanism: Determine whether data collection will be undertaken using surveys, interviews, focus groups, a combination, etc.
 - Consider value of mixed methods: Qualitative mechanisms, which enable respondents to speak to experiences in their own words, offer a more textured picture of barriers and need.
 - Timing and duration: Establish when and for how long data collection will take place.
4. **Prioritize inclusivity and the ‘do no harm principle’ in implementation:**
 - Commit funding to resource capacity building related to cultural sensitivity and inclusivity for those administering surveys.
 - Select outreach and collection tools that remove barriers for particular populations (e.g., language, accessibility) and which encourage participation (e.g., cell phones).
 - Consider diversity and representation in selecting interviewers.
 - Apply informed consent, ensure equity purpose is emphasized throughout interview.

Potential Barriers

- Low uptake or response from individuals with the largest barriers to participation (and housing)
- Data are limited to capturing a point in time, with moving targets

Practice-Based Recommendations

- CMHC should ensure that community organizations and the priority groups they represent are actively engaged in shaping data collection tools, questions, and implementation.
- CMHC should adopt a mixed methods approach, whereby both quantitative data and qualitative information (and narratives) are gathered and considered in a complementary way—with stories providing important context for quantitative data.

Accountability	Participation	Intersectionality	Sovereignty
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4. Interpreting Results; Reporting Back

Overview: Much of the harm perpetuated through data stewardship, and in research more broadly, occurs during the analysis, interpretation, and dissemination phase. It is therefore vital that the communities about whom data and information is collected are closely involved as advisors on and contributors to this stage of the process. This phase also focuses on the additional steps researchers must take—including grounding analyses in best practices in intersectionality, and developing accessible dissemination and reporting strategies connected to the strategic purpose—to embed equity and harm reduction at this key stage.

Key Elements

1. **Enlist community perspectives to advise on analysis, interpretation, and dissemination of findings:**
 - Consult community members to contextualize analysis, identify potential impacts of, dissemination, develop plan for mitigating against harm through publication of results.
 - Invest in community capacity regarding data analysis and further community-led research.
2. **Integrate intersectional principles to interpret results of data collection:**
 - Ground work in reflexivity: At the outset of and throughout the analysis, researchers should reflect on their own positionality and how it affects their assumptions, analysis, and interpretation of data results. This should also be communicated in published findings.
 - Consider intersections of identity or social locations within data.
 - Use appropriate comparison groups to contextualize findings; that is, avoid perpetuating the notion that White/cis gender/middle-class outcomes are the standard. It is often more useful to consider differences and experiences—and thus intersections—within subsets of a NHS priority group, rather than to compare priority groups and aggregate outcomes.
 - Elevate context: Situate analysis and conclusions within critical frameworks and other information to offer an account of how findings reflect how experience and housing outcomes play out within and are mediated by systems and structures of power, and are linked to broader context and local dynamics. That is, maintain focus on data providing a picture of the effects of systems, rather than individual characteristics and deficiencies.
3. **Create a dissemination strategy for findings and associated actions:**
 - Revisit purpose: Link findings to achievement of overarching purpose established at outset.
 - Identify explicit actions: Determine actionable programmatic, policy, and practice changes that have the potential to rectify issues and disparities highlighted in the data.
 - Ensure reach: Partner with communities to develop communications using a variety of tools, suitable for multiple audiences (e.g., reports, dashboards, storytelling, data walks, videos).
 - Communicate limitations of the data

Potential Barriers

- Investment—of both resources and time—is required to build community capacity in data analysis and reporting, and must be weighed against the need for timely publication of results.
- Falling short in managing the tension between the value of data and evaluation, and the risk of reinforcing inequalities, will significantly undermine equity goals of the project overall.

Practice Based Recommendations

- CMHC should engage in analysis experts in intersectional and other critical research methods.
- CMHC should invite communities to collaborate in data analysis and reporting, including by actively engaging community members to share narratives and shape dissemination tools.
- CMHC should provide communities the space to respond to findings on their own terms.
- CMHC should publicize in multiple formats for a range of audiences the findings of the collection and analysis, as well as information on the analytical process undertaken.

Accountability	Participation	Intersectionality	Sovereignty
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5. Determining Future Management, Use, and Destruction

Overview: The final phase of the framework involves determining how and by whom data will be managed, used, and maintained in the long term. Making these decisions with intention and in collaboration with communities can lay a foundation from which to work towards data sovereignty. This stage also requires that those leading stewardship efforts reflect on and undertake evaluations of their approaches, with the goal of uncovering and making public shortcomings, promising practices, and opportunities for future endeavours.

Key Elements:

1. **Set out a long-term data management strategy:**
 - Determine where/by whom data will be held, as well as access principles (e.g., for researchers, others).
 - Prevent data from languishing: Identify plan for future use and management of data, beyond the initial purpose for which it was collected, grounded in equity and harm prevention.
 - Data sovereignty: Ensure data can be accessed by—and where desired held by—the communities it concerns.
2. **Develop guidelines and a process for data destruction.**
 - Enable data destruction: Make clear to individuals that they are able to request that their data is destroyed following collection.
3. **Evaluate the process:**
 - Conduct an impact assessment, asking whether the work mitigated, worsened, ignored inequalities and structural barriers.
 - Solicit feedback: Engage community and the public on overall process, as well as dissemination/communication approach.
 - Reflect: Evaluate findings, shortcomings, and promising practices to present guidelines for future efforts.

Potential Barriers

- Moving towards data sovereignty (and thus communities owning, holding, and managing data which concerns them) requires infrastructure and capacity that is not currently present within many communities, and thus full realization of this aim will take time and resources.

Practice-Based Recommendations

- CMHC should, in collaboration with communities on whom data and information has been collected, develop a strategy for the secure management, use, and destruction of data.
- CMHC should enter into information management and data sharing agreements with communities, particularly Indigenous groups.
- CMHC should conduct (or commission) a comprehensive evaluation of its data stewardship process for the project at hand, as well as publish a corresponding report outlining a set of best practices, barriers, and shortcomings that can be used to guide future data collection and program evaluation work from an equity standpoint.

Accountability

Participation

Intersectionality

Sovereignty

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